UNITY HEALTH | EXECUTIVE COMMAND CENTER

Network-Wide Readmission & Financial Performance

Last Refresh: Feb 03, 2026
Model Status: GRADIENT BOOSTING (ACTIVE)

Fiscal Year

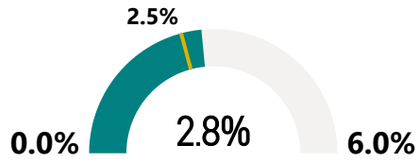
All

Total Revenue

$1,283.2M

▲ 33.6% QoQ

Re-Admission Rate



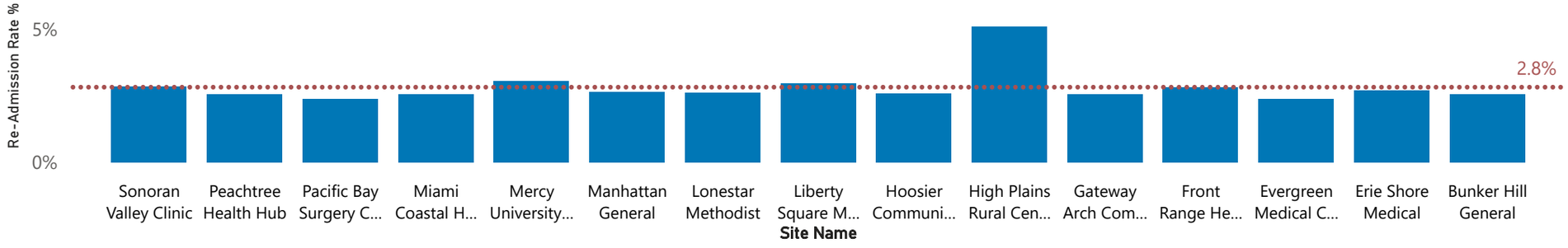
Preventable Risk Cost

$37.0M

Quarter

- Qtr 1
- Qtr 2
- Qtr 3
- Qtr 4

Facility Performance Benchmark



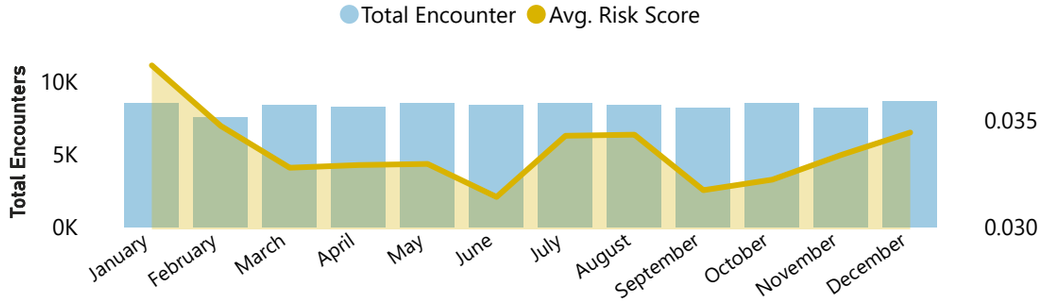
Facility Type

- Community Hospital
- Specialized Surgery Cen...
- Teaching Hospital

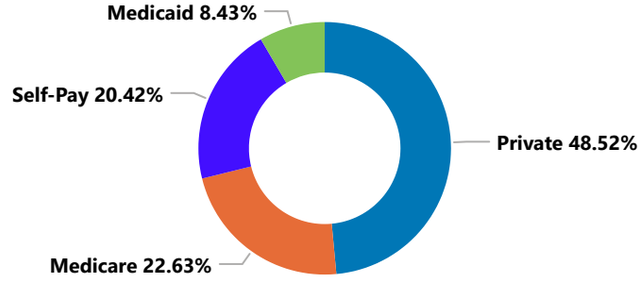
Payer Type

- Medicaid
- Medicare
- Private
- Self-Pay

Risk Wave Trend



Payer Risk Mix



Patient Watchlist

Clear all slicers

Note: Reduce Readmission Rate below 3.4% by Q4. Highlighted 'Danger Zones' represent priority areas for immediate clinical audit and discharge protocol review.

CLINICAL RISK STRATIFICATION & AI PREDICTION

Gradient Boosting Model

Last Refresh: Feb 03, 2026
Model Status: GRADIENT BOOSTING (ACTIVE)
Prediction Confidence: 97%

Fiscal Year

All

Facility Type

- Community Hospital
- Specialized Surgery Cent...
- Teaching Hospital

Clinical Specialty

- Cardiology
- Endocrinology
- Gastroenterology

Risk Threshhold

0.70

Average Risk Probability

3.4%

High Risk Patients

1419

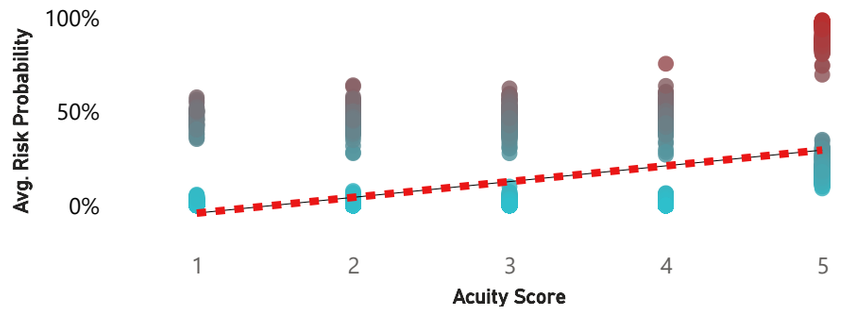
Model Accuracy

97.5%

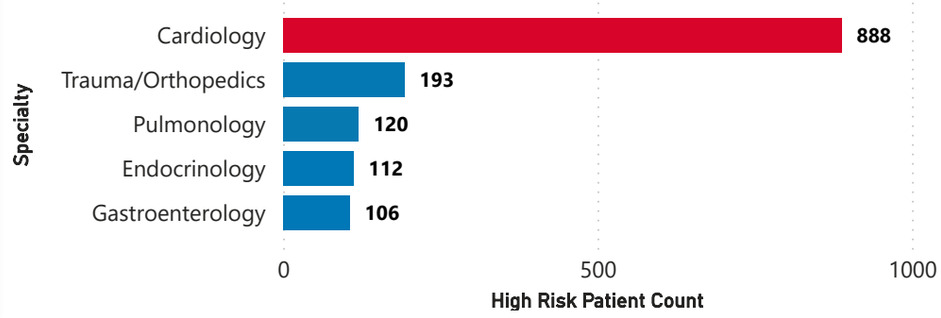
Diagnosis Risk Concentration

Trauma/Orthopedics	Gastroenterology	Endocrinology	Pulmonology	Cardiology
30,376	19,950	19,874	19,851	9,949

Risk Vs. Acuity Correlation



High-Risk Concentration by Specialty



Executive Overview

Patient Intervention Watchlist

Clear all slicers

Note: Specialties highlighted in Red in the Heatmap require immediate discharge review.
Use the Risk Threshold slider to prioritize the top 5% of cases.

PATIENT INTERVENTION WATCHLIST: HIGH PRIORITY TARGET

High-Priority target For Discharge & Follow-Up

Model Status: GRADIENT BOOSTING (ACTIVE)

Prediction Confidence: 97%

Select Date

2025

January

1

Priority Patients

2

Average Severity

3

Active Bed Count

1560

EHR ID

US-128311

Facility Type

- Community Hospital
- Specialized Surgery Cent...
- Teaching Hospital

Clinical Specialty

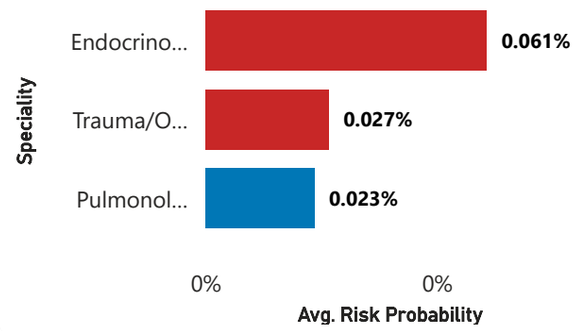
- Cardiology
- Endocrinology
- Gastroenterology
- Pulmonology
- Trauma/Orthopedics

Clinical Risk Detail Table

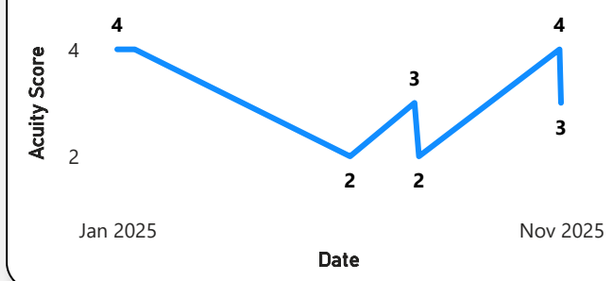
Encounter ID	EHR ID	Acuity Score	Readmission Risk Score	Specialty	Predicted Action
107172	US-128311	4	0.00	Trauma/Orthopedics	Standard Protocol
109011	US-128311	2	0.00	Pulmonology	Standard Protocol
156884	US-128311	4	0.00	Trauma/Orthopedics	Standard Protocol
168230	US-128311	2	0.00	Trauma/Orthopedics	Standard Protocol
177562	US-128311	4	0.00	Endocrinology	Standard Protocol
193688	US-128311	3	0.00	Trauma/Orthopedics	Standard Protocol
196603	US-128311	3	0.00	Trauma/Orthopedics	Standard Protocol
Total					

Last Refresh:

Risk Factor Analysis



Patient History Trend



Executive Overview

AI Model Governance & Prediction Audit

Clear all slicers

Note: *CONFIDENTIAL PATIENT DATA: This watchlist is for clinical decision support only and does not replace professional medical judgment. Risk scores are based on historical EHR patterns and AI forecasting. Data Refresh: [Last Refresh Date] | Model Version: GB-v2.1*

AI MODEL GOVERNANCE & PREDICTION AUDIT

High-Priority Target for Discharge Planning & Follow-up

Last Refresh: Feb 03, 2026
Model Status: GRADIENT BOOSTING (ACTIVE)
Prediction Confidence: 97%

Year

All

Total Predictions

89099

Avg Confidence Score

0.034

False Positive rate

0.003

Month

- January
- February
- March

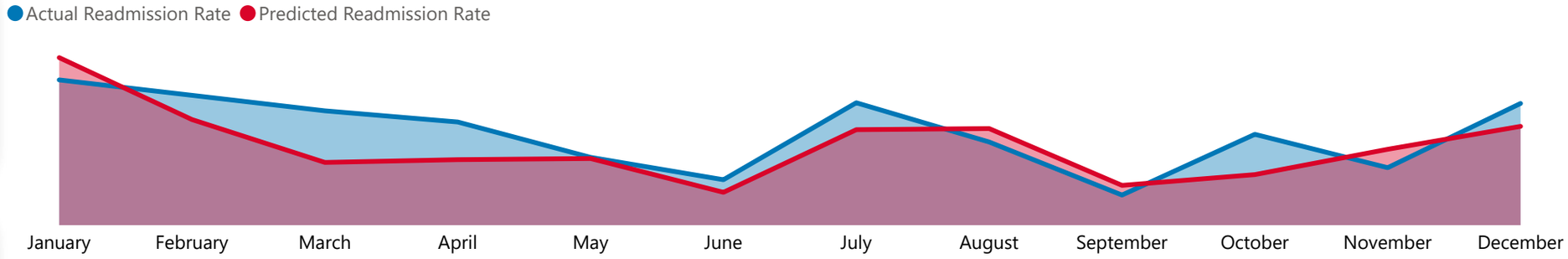
Day

- 1
- 2
- 3

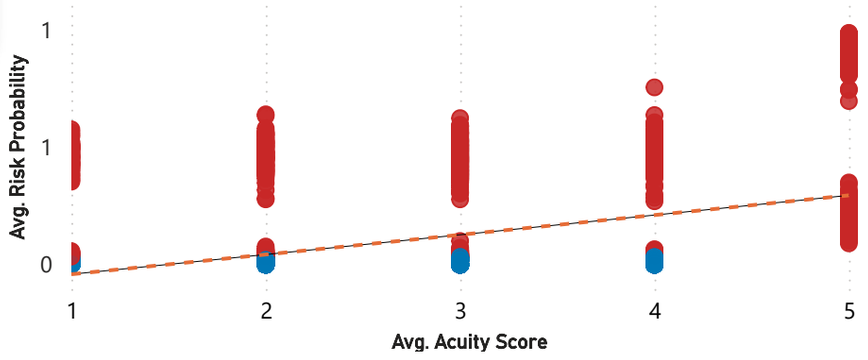
Facility Type

- Community Hospital
- Specialized Surgery Cent...
- Teaching Hospital

Model Performance Trend (Confidence vs. Accuracy)



Risk Score Distribution



High-Deviation Audit Log

Encounter ID	EHR ID	Acuity Score	Readmission Risk Score
177128	US-934148	5	0.9862
172805	US-738892	5	0.9862
147920	US-545997	5	0.9836
123872	US-238197	5	0.9827
104113	US-589478	5	0.9826
184809	US-992006	5	0.9825
113962	US-128227	5	0.9825
157907	US-865786	5	0.9819
174663	US-368241	5	0.9818

Executive Overview

Clear all slicers

Note: This page is used for algorithmic bias monitoring and model drift detection. Model GB-v2.1 underwent last training cycle on March 01, 2026. All deviations are reviewed by the Unity Health Quality Committee.